Weekly paclitaxel · relapsed or refractory small-cell lung cancer (SCLC)

Treatment scope · NCCP 00226 NCCP 00226d: small-cell lung cancer that has returned or has not responded to previous treatment. This use is outside the Irish licence and requires the hospital off-label approval and consent process. This guide is for conventional paclitaxel alone.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Paclitaxel · lung supportive care

Active medicines: Paclitaxel

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- National Cancer Control Programme (NCCP) first-dose suggested premedication: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before infusion.
- From Day 8, NCCP permits omission of routine dexamethasone; previous reaction or higher risk requires consultant-directed adjustment. Famotidine omission is also a consultant/local-policy decision with close observation.
- The actual conventional product SmPC uses a different generic premedication schedule. Reconcile the exact product and local protocol; no automatic patient-led omission or switch to nab-paclitaxel.
- Consider analgesia for muscle/joint aches; review neuropathy before every dose and growth-factor support when indicated.
- Low emetic risk: prevention and rescue medicines remain separate from reaction premedication. Confirm individual suitability, interactions and duration; this reference is not a prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00226 v9 · p4 · NCCP NCIS medical-oncology antiemetics V8, low risk

Sources checked 2026-10-29 · SACTCheck v0.84.25 · Regimen-level drafts; prescriber and oncology-pharmacy verification required